



Meaning Bridge — Replit Agent Build Prompt (MVP)

How to use this: Paste **Section A** as your first message to Replit Agent to scaffold the app. Keep **Sections B-H** in this file (or your Replit `replit.md`) as the spec the Agent and you refer back to. Build the **MVP scope** first; everything under *Later / v2* is deferred — but design so it slots in.

Enforce on every screen: *bridge, not replacement.* Meaning Bridge augments therapists and human connection; it never replaces them, and it hands off to human care when a screener crosses its clinical cut-point. Or, in Dr. Neimeyer's words: *respect resilience — most mourners need understanding, not professional treatment; read the red flags and reach out to those who are clearly struggling; offer support, and when it is not enough, offer grief therapy.*

Clinical instruments are now confirmed. The GIS, GAQ, and the public-health pyramid below are taken from the published, validated, public-domain sources (Lee & Neimeyer, *Death Studies* 2022 & 2025; Neimeyer/IWG 2025). Use the exact items, scales, and cut-scores given here. The GIS and GAQ are explicitly placed in the public domain for clinical and research use with citation — no permission needed beyond citing the source articles.

SECTION A — Paste this into Replit Agent first

Build a web app called "Meaning Bridge" – an AI-assisted grief-support platform that meets bereaved people at the level of support they need and AUGMENTS therapists rather than replacing them. It's grounded in Dr. Robert Neimeyer's meaning-oriented, continuing-bonds approach and a validated public-health model of grief.

Brand: name "Meaning Bridge"; logo is an infinity symbol formed by two bridges (file provided). Palette: deep navy (#1B3A5B) to teal (#2E8B8B) gradient, warm off-white background, generous whitespace. Calm, humane, credible – never clinical-cold, never a chatbot gimmick. Use the infinity-bridge logo in the header and on the landing page.

Tech stack:

- React + Vite + TypeScript frontend, Tailwind CSS
- Node/Express backend
- PostgreSQL (use Replit's built-in Postgres) with Drizzle ORM
- Auth with two roles: Seeker (bereaved user) and Practitioner (therapist)
- AI conversation via the OpenAI/Anthropic API – read the API key from Replit Secrets (env var, never hard-coded). If no key is set, stub the AI with a safe canned reply.
- QR generation client-side with the `qrcode` npm package (no external QR service).

Build this MVP, in this order:

1. Auth + the two roles + a Postgres schema (see data model in the spec).
2. SEEKER onboarding: a warm, first-name welcome (NOT a generic "Welcome to Meaning Bridge" loop), then the GIS screener (the 5-item Grief Impairment Scale, exact items and 0-4 scale provided in the spec), auto-scored. Route the user into one of three care tiers by GIS total score: Universal (Grief Literacy), Targeted (Enhanced Support), or Clinical (Specialist). The validated GIS cut score is ≥ 9 = clinically significant functional impairment (route toward Clinical). Show the score as a conversation starter, never as a cold verdict.
3. A tier-aware AI grief companion (text chat). Inject the system prompt from the spec, with the user's first name and tier. Each tier has its own tone and what it surfaces.
4. An always-on self-harm safety layer in every conversation (details in the spec). IMPORTANT: GIS item 3 (unhealthy/self-destructive coping) and any later GAQ items about wanting to die must trigger the safety layer, not be treated as ordinary scored items.
5. A private journal/history for the Seeker, a post-session feedback survey, and an email "notify me at launch" opt-in.
ALSO build a PUBLIC landing page at the route `/notify` (no login required) with a short "Meaning Bridge is coming – get notified when it launches" form (email + an optional "I'm a: Seeker / Therapist" choice) that writes to the notify_opt_ins table. On the home/landing page, render a QR CODE that encodes the absolute URL of that `/notify` page, with a caption "Scan to get notified when Meaning Bridge launches." Build the QR URL from the current origin at runtime (`\${window.location.origin}/notify`) so it works on Replit's dev and deployed domains without hard-coding. Show the plain link and a "Copy link" button under the QR, and provide a full-screen "Present" mode so it can be projected at a conference and scanned from a room.
6. PRACTITIONER side: profile with self-declared license type + the US states they're licensed in; a client roster; consent-gated session summaries the therapist reads BEFORE a live session; and surfaced trend/self-harm risk flags.

Treat all user content as sensitive health data: never put it in URLs, query strings, or logs; gate any sharing behind explicit, logged consent; do not auto-create third-party accounts or store card/bank/ID data.

Start by scaffolding the project and the database schema, then build feature 1.

After it scaffolds, feed it the system prompt (Section D), the safety layer (Section E), the GIS/GAQ items (Section C), and

Section G (QR + notify page) as it reaches each feature. Then continue feature by feature.

SECTION B — Why this exists (context for every decision)

Meaning Bridge serves three concentric audiences via one platform:

1. **Bereaved people** wanting a warm, private place to process grief.
2. **Therapists** who are *intrigued and terrified* of AI — offered a trustworthy companion that simplifies their work and extends therapy between sessions.
3. **Workshop participants** at Dr. Neimeyer's trainings, who become a recurring feedback/focus group.

It is a **bridge between sessions** (continuity of care) and a **bridge between worlds** (continuing bonds with the deceased). The logo — an infinity symbol formed from two bridges — carries exactly this dual meaning.

SECTION C — The clinical spine: Public-Health Pyramid + validated screeners

C.1 The pyramid (Aoun et al.; Neimeyer / IWG 2025)

Every Seeker is routed into one tier at onboarding by the GIS. The model is non-pejorative: **most mourners are resilient and need understanding, not treatment**. Tier names below use the public-health labels (with Dr. Neimeyer's "levels of service" in parentheses); use these names in code/config.

Tier (level)	~Share	Population	Service offered	App response
Universal — Grief Literacy (Level 1)	~60%	General bereaved (+ supporters, caregivers)	Informal support; information about services; psychoeducation; community grief literacy	Conversational companion focused on validation, normalization, gentle continuing-bonds conversation. Light UI, low-frequency check-ins.
Targeted — Enhanced Support (Level 2)	~30%	At-risk profiles (e.g. suicide/overdose-loss survivors, bereaved parents, violent/non-normative loss)	Counselling support; mutual-help groups; crisis & spiritual support	Structured check-ins, guided prompts, closer monitoring, gentle nudges toward mutual-help/counselling.
Clinical — Specialist (Level 3)	~10%	Complications: PGD, PTSD, MDD, functional impairment	Specialist therapies; mental-health services; tailored/protocol interventions	Warmly and consistently encourages live therapy; prominent referral path; most sensitive risk monitoring.

Store tier names and % as configurable constants. Note the validated figures are **60 / 30 / 10**, not 40/30/10. Routing is driven by the GIS score (and later the GAQ/ISLES), not by self-selection alone.

C.2 Grief Impairment Scale (GIS) — primary MVP screener

5 items. **Instructions to user:** *"Using the scale below, please indicate how often (in the past 30 days) you had difficulty functioning because of your grief. We define 'grief' as your reactions to your significant loss."*

Response scale (per item): 0 = 0 days (never) · 1 = 1–3 days (seldom) · 2 = 4–15 days (occasionally) · 3 = 16–29 days (often) · 4 = 30 days (always).

Items (each with example sub-bullets shown to the user as helper text):

1. **Experienced problems with thinking because of your grief** — e.g. difficulty with attention/concentration; memory problems; difficulty making decisions.
2. **Experienced health problems because of your grief** — e.g. sickness, pain, or discomfort; sleep disturbances; low energy/exhaustion.
3. **Engaged in unhealthy activity to cope with your grief** — e.g. alcohol/substance use; unhealthy eating (over- or under-eating); self-destructive behavior. (*SAFETY-LINKED — see Section E. A non-zero score here, especially 3–4, should be treated as a risk signal, not just scored.*)
4. **Unable to fulfill an important responsibility in life** (work/school, housekeeping, caring for others) **because of your grief** — e.g. absence/poor performance; unkept home; unable to care adequately for dependents.

5. **Unable to positively engage with others because of your grief** — e.g. avoiding a significant person/place/event; conflict with others; being treated hurtfully by others.

Scoring: sum the 5 items (range 0–20). Higher = greater grief-related functional impairment.

Validated cut score: ≥ 9 = clinically significant functional impairment (79% sensitivity, 74% specificity; AUC ≈ 0.88).

MVP routing rule (configurable):

- GIS **0–4** → **Universal**
- GIS **5–8** → **Targeted**
- GIS ≥ 9 → **Clinical** (prominent encouragement toward live therapy + referral path)
- Any item-3 or item-5 response of 3–4, or any safety signal in chat → escalate per Section E regardless of total.

Source: Lee & Neimeyer (2022), *Death Studies*, "Grief Impairment Scale." Public domain; cite the article. Use it as a **conversation starter**, not a label ("shift from numbers to narratives").

C.3 Grief Attack Questionnaire (GAQ) — v1.1 / optional module

16 items, 4 subscales (4 items each): **Panic** (1–4), **Yearning** (5–8), **Disorganization** (9–12), **Despair** (13–16). **Severity scale:** 0 = not at all · 1 = slightly · 2 = somewhat · 3 = quite a bit · 4 = overwhelmingly. **Instructions:** "Think of your most recent experience with a 'grief attack' — a sudden, often unexpected upwelling of grief-related anguish. Indicate how intensely you felt each of the following during your most recent grief attack."

Total score range 0–64. Validated cut score: ≥ 35 indicates a grief attack associated with impairing levels of grief (88% sensitivity, 67% specificity; AUC ≈ 0.80).

Items: Panic — (1) trembling/shaking, (2) sweating, (3) numbness/tingling, (4) dizzy/lightheaded/faint. Yearning — (5) thoughts about the deceased, (6) longing/yearning, (7) emotional pain (anger/bitterness/sorrow), (8) irresistible sobbing/crying. Disorganization — (9) unreality/detachment from self, (10) fear of dying, (11) fear of losing control or "going crazy", (12) confusion about role/identity. Despair — (13) life is meaningless without the deceased, (14) wanting to die rather than live without the deceased, (15) felt alone/lonely without the deceased, (16) trouble believing the deceased is really gone.

SAFETY-LINKED: item 14 ("wanting to die rather than live without the deceased") must route to the Section E safety layer, never be silently scored. Source: Lee & Neimeyer (2025), *Death Studies*, "Grief Attack Questionnaire." Public domain; cite the article. Offer the GAQ when a Seeker reports sudden, intense surges of grief — it points toward a dedicated "grief attacks" module (psychoeducation, paced breathing, grief-focused mindfulness, continuing-bonds practices).

C.4 ISLES (Integration of Stressful Life Experiences Scale) — v2 / meaning module

6 items; two subscales — **Comprehensibility** (sense-making of the event) and **Footing in the World** (personal identity). Has a cut score for a "crisis of meaning" associated with functionally impairing grief. Reserve for the meaning-oriented module in v2.

SECTION D — AI companion system prompt (paste into the model call)

You are the Meaning Bridge companion, a warm, humanly-oriented presence that supports people through grief, grounded in Dr. Robert Neimeyer's meaning-oriented, continuing-bonds approach and a public-health understanding of grief.

Your role is to ACCOMPANY, not to treat. You are a bridge between sessions and a bridge to continuing bonds – never a replacement for a therapist or for human connection. Respect resilience: most grieving people need understanding, not treatment.

Voice: calm, unhurried, kind, plainspoken. Greet {{first_name}} warmly. Validate before you guide. Reflect feeling without amplifying distress. Ordinary language, not jargon. Treat any assessment scores as conversation starters, never as labels or verdicts.

Behavior by tier ({{tier}} = universal | targeted | clinical):

- universal (Grief Literacy): validation, normalization, psychoeducation, and gentle continuing-bonds conversation. This person likely needs understanding, not treatment.
- targeted (Enhanced Support): add structured, gentle prompts and check-ins; suggest mutual-help and counselling resources; watch more closely; nudge toward additional human support when rough patches persist.
- clinical (Specialist): warmly and consistently encourage live therapy and surface the referral path; emphasize that a person can help here. Do not attempt to deliver treatment.

Continuing bonds: with care and at the person's lead, help them stay in relationship with the one they lost (e.g. things they'd want to say; symbolic correspondence; rituals of remembrance). Follow their pace; never push a re-experiencing they did not ask for.

Hard limits:

- Never claim to be human, a therapist, or a substitute for one.
- If the person expresses thoughts of self-harm, suicide, wanting to die, or self-destructive coping, STOP normal flow, respond with care, and surface crisis resources. Never name or describe methods of self-harm.
- Do not diagnose. Do not state clinical cut-scores to the user, or give medical/legal/financial advice.
- Do not induce dependence on you; encourage the person's real-world supports.
- Keep the person's content private; never share a summary without their logged consent.

SECTION E — Safety layer (non-negotiable, all tiers)

- Continuously scan Seeker messages **and screener responses** for self-harm / suicide / self-destructive signals. Specific structured triggers: **GIS item 3** (unhealthy/self-destructive coping) scored $\geq 2-3$, and **GAQ item 14** ("wanting to die...") scored ≥ 1 . These route to the safety layer; they are never "just scored."
- On detection: respond with care, do **not** continue normal flow, surface crisis resources, and (for Practitioner-linked Seekers, with consent) escalate to the therapist via a `safety_event`.
- **Never name or describe self-harm methods** anywhere, including in any "things to remove" guidance.
- Never position the AI as a substitute for professional or human support.
- Honor the **cut-point hand-off**: GIS ≥ 9 (and GAQ ≥ 35 where used) routes toward human care rather than deepening AI reliance.
- Practice clinical humility: assessment can identify *what* is wrong but not *how* to fix it — so the app's job at the high end is to connect, not to treat.
- Log safety events for consent-gated clinician review only — never publicly.

SECTION F — Data model (Drizzle / Postgres starter)

- **users**: id, role (`seeker` | `practitioner`), first_name, email, auth fields, created_at

- **practitioner_profiles**: user_id, license_type, licensed_states (string[]), bio, photo_url, accepting_clients (bool)
- **grief_profiles**: user_id, tier (universal | targeted | clinical), deceased_relationship (nullable), cause_of_death (nullable), time_since_loss (nullable), pet_loss (bool)
- **screeener_results**: id, user_id, instrument (GIS | GAQ | ISLES), item_responses (jsonb), score, subscale_scores (jsonb, for GAQ), cut_point_flag (bool), tier_assigned, completed_at
- **conversations / messages**: id, user_id, role, content, risk_flags (jsonb), created_at
- **session_summaries**: id, seeker_id, practitioner_id, content, consent_granted (bool), consent_at, created_at
- **safety_events**: id, seeker_id, source (chat | gis_item3 | gaq_item14), severity, handled (bool), created_at
- **consents**: id, seeker_id, scope, granted (bool), granted_at, revoked_at
- **feedback_responses**: id, source (workshop | app), rating, comments, created_at
- **notify_opt_ins**: id, email, role_interest (seeker | practitioner), source, created_at

Config constants (all in one place): tier names + percentages (60/30/10); GIS items + 0-4 scale + cut ≥ 9 + routing bands (0-4 / 5-8 / ≥ 9); GAQ items + subscales + cut ≥ 35 ; ISLES (v2). Cite the source articles in code comments.

SECTION G — Launch sign-up QR code + notify landing page (build in feature 5)

Purpose: let people at Dr. Neimeyer's keynotes/workshops scan a projected QR code, land on a sign-up page, and leave their email to be notified when Meaning Bridge launches. This also powers the early-interest list and future funding evidence.

The /notify landing page (public, no login):

- The infinity-bridge logo, headline "Meaning Bridge is coming.", and a one-line description.
- Form: email (required) + optional single-choice "I'm a: Seeker / Therapist". On submit, write to `notify_opt_ins`; show a calm confirmation ("Thank you — we'll let you know when it's ready.").
- Read an optional `?src=` event label (e.g. `/notify?src=chicago-keynote`) into `source` so opt-ins are attributable; default `source` to `qr`. No personal data in the URL — `src` is just an event label.
- Validate the email; handle duplicates gracefully ("you're already on the list").

The QR code (on the home/landing page, reusable component):

- Generate **client-side** with the `qrcode` npm package — no external QR image service (privacy + reliability).
- Encode the absolute URL built at runtime: ``${window.location.origin}/notify?src=qr``. Must work unchanged on Replit's dev URL and the deployed domain — never hard-code the host.
- Render as crisp SVG or high-res canvas ($\geq 300 \times 300$ px, quiet-zone margin, error-correction level "H") so it scans from a projector screen.
- Below the QR: the plain clickable link + a "Copy link" button.
- A **"Present" / full-screen mode** (e.g. `/present`) showing just a very large QR on a clean background with the caption — readable/scannable across a conference room.

Acceptance check: scanning the projected QR opens `/notify`, submitting an email stores a row in `notify_opt_ins`, and the list is exportable. Must resolve on the live deployed Replit domain.

SECTION H — Ship the notify page FIRST (keynote fallback)

There is a hard near-term deadline: Dr. Neimeyer leaves Wednesday for a Chicago keynote. If the full MVP isn't deployed and stable in time, **deploy ONLY the /notify landing page + QR code first**, as a standalone page, so the keynote sign-up works regardless of the rest. The QR points at the live `/notify` URL. Everything else (screeener, companion, practitioner portal) can follow. Build the notify page so it stands alone with no dependency on auth or the AI.

Later / v2 — do NOT build now; design so it slots in

- **Modules** (composable): art therapy; binaural-beats / theta-wave & sound; body-integration & guided meditation; movement/exercise (incl. Tai Chi); daily-living & meal-planning; **grief-attacks module** (driven by the GAQ — psychoeducation, paced breathing, grief-focused mindfulness); meaning module (driven by the ISLES); continuing-bonds workbook; integrated **bookstore** for Dr. Neimeyer's ~50 books.
- **GAQ and ISLES** as added screeners/modules (items already specified in Section C).

- **Wearable/biometric integration** (Oura, smartwatches): HRV & related markers, consent-gated.
 - **Realistic avatar welcome** (NVIDIA "James"-style). *Decision pending* — Dr. Neimeyer is cautious about leading with a personal-likeness avatar; start with a neutral, inclusive default, treat the avatar as opt-in later.
 - **Therapist match-making**: directory filtered by license/state; first-appointment scheduling end-to-end.
 - **Pet-grief** track (canine/feline).
 - **Multilingual**: Spanish first (EK / Instituto de Terapia de Luto, Mexico collaboration).
 - **Pricing**: therapist subscription (larger audience) + client subscription (smaller); possible tier-mapped pricing; module add-ons; explore FSA/HSA eligibility of the subscription fee. Keep affordable.
-

Definition of done (MVP)

A real bereaved person and a real therapist can each sign in; the Seeker completes the GIS, is routed to the right tier (Universal / Targeted / Clinical) by the validated cut-scores, and holds a safe, humane conversation; safety triggers (GIS item 3, chat signals) escalate correctly; and — with logged consent — a summary reaches their therapist. The team can put it in front of workshop participants and collect feedback. The `/notify` page + QR can ship first as a standalone keynote teaser.

Clinical content (GIS, GAQ, ISLES, pyramid) is drawn from the validated, public-domain sources: Lee & Neimeyer, Death Studies (2022, 2025); Aoun et al. (2012, 2015); Neimeyer / IWG Grief Therapy Workgroup (2025). The GIS and GAQ are public domain for clinical and research use with citation. Set the AI API key in Replit Secrets — never commit it.